10 PRINTABLE REFERENCE CHARTS

Everything on the fridge door for calmer glucose days.

A matched set of 8" × 11" educational charts — targets, lows, highs, carbs, plate method, A1C, insulin timing, sick days, and foot care — designed to look polished enough to sell and clear enough to use daily.

01 Blood Glucose Target Ranges
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8 × 11 IN | PRINT-READY PDF | COHESIVE BRAND SET

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Blood Glucose Target Ranges

Typical adult targets used in diabetes education. Your care team may set different goals based on age, pregnancy, hypoglycemia risk, or other conditions.

80–130	<180	90–150	<7%
BEFORE MEALS MG/DL	1–2 HR AFTER MEALS MG/DL	BEDTIME MG/DL	A1C GOAL (MANY ADULTS)

When to check & what "in range" often means

TIMING	TYPICAL TARGET	NOTES
Fasting / before meals	**80–130 mg/dL**	ADA general adult guidance
1–2 hours after starting a meal	**< 180 mg/dL**	Peak post-meal reading
Bedtime	**90–150 mg/dL**	Ask your clinician for your range
Before exercise	**100–250 mg/dL***	*Avoid intense exercise if high with ketones
During illness	**Check more often**	See Sick Day Rules chart

Low — treat now

Below 70 mg/dL HYPOGLYCEMIA

- Treat with 15g fast-acting carbs
- Recheck in 15 minutes
- Repeat if still under 70

High — take action

Above 250 mg/dL CHECK KETONES

- Drink water; follow care plan
- Test for ketones if instructed
- Call clinician if rising or ill

Personalize your numbers

Older adults, young children, people with frequent lows, or those who are pregnant often have different targets. Write your clinician's goals here:

Fasting _____ · After meals _____ · Bedtime _____ · A1C _____

Educational reference only — not medical advice. Follow the targets set by your healthcare provider. Based on commonly cited adult ADA education ranges.

Hypoglycemia Quick Action

Low blood sugar (<70 mg/dL) can escalate quickly. Keep this chart where you check glucose — and teach household members the same steps.

Early warning signs

- Shakiness, sweating, clammy skin
- Fast heartbeat, anxiety
- Hunger, irritability
- Tingling lips or tongue
- Mild confusion or dizziness

Severe — get help now

- Inability to swallow safely
- Seizure or loss of consciousness
- Extreme confusion / combativeness
- Cannot treat themselves

Do not force food or drink. Use glucagon if prescribed and call emergency services.

The 15–15 Rule

1. **Check** — Confirm glucose is under 70 mg/dL (treat if symptomatic and you can't check).
2. **Treat** — Take **15 grams** of fast-acting carbohydrate.
3. **Wait 15 minutes** — Recheck. If still <70, repeat another 15g.
4. **Follow up** — Once stable, eat a snack with protein/carbs if your next meal is more than an hour away.

Fast-acting 15g carb options

LIQUIDS

- 4 oz (½ cup) juice
- 4 oz regular soda
- 1 tbsp honey or syrup

TABLETS / GELS

- 3–4 glucose tablets
- 1 tube glucose gel
- Follow package labeling

OTHER

- ~6–7 hard candies*
- 1 tbsp sugar
- *Not chocolate — fat slows absorption

If someone is unconscious or can't swallow

Call emergency services. Give glucagon per training (injection or nasal). Place person on their side after glucagon. Never put food, juice, or candy in an unconscious person's mouth.

Hyperglycemia Warning Signs

High blood sugar builds more slowly than lows — but untreated highs can lead to dehydration, ketones, and emergency conditions like DKA or HHS.

>180
ABOVE TARGET
MG/DL

>250
CHECK KETONES
MG/DL

>300
URGENT IF RISING
OR WITH ILLNESS

Common high symptoms

- Increased thirst & dry mouth
- Frequent urination
- Blurred vision
- Fatigue / headache
- Hunger or nausea
- Slow-healing cuts

Possible ketone / DKA signs

- Fruity or acetone breath
- Deep, rapid breathing
- Vomiting, severe nausea
- Stomach pain
- Confusion or drowsiness
- Inability to keep fluids down

What to do when high

1. Confirm with a meter or sensor reading.
2. Drink water (unless restricted by clinician).
3. Take correction insulin only as prescribed.
4. Recheck in 1–2 hours; note trend.
5. If ≥250 mg/dL, check urine/blood ketones if instructed.
6. Avoid vigorous exercise if ketones are present.
7. Call your care team for persistent highs or illness.
8. Seek emergency care for DKA warning signs.

Seek emergency care if

Vomiting prevents keeping fluids down · moderate/large ketones with symptoms · difficulty breathing · confusion · glucose stays very high despite following your plan · suspected DKA or HHS.

Educational reference only — not medical advice. Correction dosing and ketone protocols are individualized. Never change insulin without clinician guidance.

Carb Counting Pocket Guide

Carbohydrates raise blood glucose the most. Counting grams helps match food to insulin or medication plans. Values are typical averages — always check labels.

15g	45–60g	15–20g
1 CARB SERVING (EXCHANGE)	COMMON MEAL STARTING POINT*	COMMON SNACK RANGE*

Typical carbs per serving

FOOD	PORTION	CARBS	FOOD	PORTION	CARBS
Bread	1 slice	**15g**	Rice, cooked	⅓ cup	**15g**
Tortilla	6-inch	**15g**	Pasta, cooked	⅓ cup	**15g**
Potato	½ cup	**15g**	Beans, cooked	½ cup	**15–20g**
Milk	1 cup	**12g**	Yogurt, plain	¾–1 cup	**12–15g**
Apple / orange	1 small	**15g**	Banana	½ large	**15g**
Grapes	17 small	**15g**	Fruit juice	½ cup	**15g**
Cereal, dry	~¾ cup	**15–30g**	Oatmeal, cooked	½ cup	**15g**
Crackers	4–6	**15g**	Ice cream	½ cup	**15–20g**

Usually little / no carb

- Non-starchy vegetables (most)
- Meat, poultry, fish, eggs
- Cheese, oils, butter
- Nuts & seeds (watch portions)
- Water, black coffee, unsweet tea

Label reading tips

- Use **Total Carbohydrate**, not just sugar
- Multiply by the servings you eat
- Fiber & sugar alcohols: ask your educator how you count them
- “Net carbs” marketing varies — verify math

*Meal/snack targets vary widely. Work with a dietitian for your insulin-to-carb ratio or medication plan.

Glycemic Index Food Guide

Glycemic index (GI) ranks carb foods by how quickly they raise blood glucose. Lower GI choices often produce a gentler rise — but portion size (glycemic load) still matters.

Low GI ≤55

- Steel-cut / rolled oats
- Barley, bulgur, quinoa
- Most beans & lentils
- Non-starchy vegetables
- Apple, berries, orange
- Greek yogurt, milk
- Nuts, peanuts
- 100% stone-ground bread*

Medium GI 56–69

- Brown rice
- Couscous
- Whole-wheat bread*
- Quick oats
- Sweet potato
- Banana (ripe)
- Pineapple
- Popcorn

High GI ≥70

- White bread, bagels
- White rice, rice cakes
- Instant potatoes
- Cornflakes / puffed cereals
- Pretzels, crackers
- Watermelon
- Sugary drinks
- Many baked desserts

Practical ways to lower the rise

- Pair carbs with protein, fat, or fiber
- Choose intact grains over refined
- Prefer whole fruit over juice
- Cook pasta al dente; cool & reheat rice/pasta when helpful
- Watch portions even on low-GI foods
- Vinegar or lemon with meals may blunt spikes for some
- Walk 10–15 minutes after eating
- Compare your CGM/meter — personal responses vary

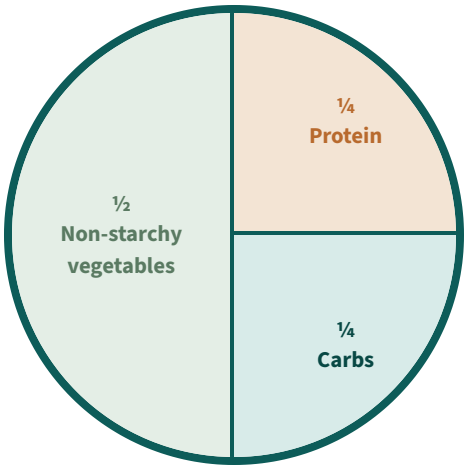
GI is a tool, not a rulebook

Ripeness, cooking method, processing, and what else is on the plate change GI. Total carbs and your medication plan remain the foundation. *Bread GI varies widely by brand and grain structure.

Educational reference only — not medical advice. GI values are approximate published averages and can vary by source and preparation.

Diabetes Plate Method

A simple visual for building balanced meals without weighing every gram. Use a 9-inch plate. Adjust portions with your dietitian if you use insulin dosing or specific carb goals.



Add water, unsweet tea, or milk as planned · Include healthy fats in moderation

½ Non-starchy veg

- Broccoli, greens, salad
- Peppers, cucumber, zucchini
- Cauliflower, green beans
- Tomatoes, mushrooms

¼ Lean protein

- Chicken, turkey, fish
- Eggs, tofu, tempeh
- Lean beef or pork
- Cheese, cottage cheese

¼ Carbohydrate

- Whole grains, pasta, rice
- Starchy veg (potato, corn)
- Beans, lentils
- Fruit or yogurt

Build-a-meal examples

MEAL	PLATE IDEA
Lunch	Salad greens + grilled chicken + quinoa
Dinner	Roasted veg + salmon + sweet potato
Breakfast*	Veg omelet + berries + toast

*Breakfast often needs a flexible plate — emphasize protein + fiber.

Extra steady tips

- Fill the veg half first
- Choose water over sugary drinks
- Use smaller plates if portions creep up
- Beans can count as protein or carb — pick one “slot”
- Still count carbs if you dose insulin

Educational reference only — not medical advice. Adapted from widely used diabetes plate education models. Individual nutrition plans differ.

A1C & Average Glucose

A1C estimates your average blood glucose over ~2–3 months. Estimated average glucose (eAG) translates A1C into the same units as your meter.

<5.7%	5.7–6.4%	≥6.5%
TYPICAL NONDIABETES	PREDIABETES RANGE	DIABETES DIAGNOSTIC*

A1C → estimated average glucose (eAG)

A1C %	EAG MG/DL	EAG MMOL/L	A1C %	EAG MG/DL	EAG MMOL/L
5.0	97	5.4	**8.0**	183	10.1
5.5	111	6.2	**8.5**	197	10.9
6.0	126	7.0	**9.0**	212	11.8
6.5	140	7.8	**9.5**	226	12.6
7.0	154	8.6	**10.0**	240	13.4
7.5	169	9.4	**11.0**	269	14.9

Formula often used: eAG (mg/dL) ≈ 28.7 × A1C − 46.7

Many adults' A1C goal

< 7%

Roughly eAG under ~154 mg/dL. Goals may be tighter or looser depending on hypoglycemia risk, age, and other factors.

Why meter & A1C can disagree

- A1C is a long-term average
- Anemia, kidney disease, pregnancy can skew A1C
- Recent highs/lows may not yet show
- CGM time-in-range complements A1C

Track your numbers

My last A1C: _____ % on _____ · My target A1C: _____ % · My eAG goal: _____ mg/dL · Next lab date: _____

Educational reference only — not medical advice. *Diagnosis requires clinician confirmation. eAG values follow commonly published NGSP/ADAG conversions.

Insulin Types & Timing

Knowing onset, peak, and duration helps you plan meals, corrections, and overnight coverage. Brand names vary — match this chart to what your clinician prescribed.

Action profile overview

TYPE	EXAMPLES*	ONSET	PEAK	DURATION
RAPID	Lispro, aspart, glulisine	**10–20 min**	**1–3 hr**	**3–5 hr**
SHORT / R	Regular (U-100)	**30–60 min**	**2–4 hr**	**5–8 hr**
INTERMEDIATE	NPH	**1–3 hr**	**4–8 hr**	**12–16 hr**
LONG	Glargine, detemir	**1–2 hr**	**Minimal**	**~20–24 hr**
ULTRA-LONG	Degludec, U-300 glargine	**~1 hr**	**Minimal**	**36–42+ hr**

*Examples of classes — not a prescribing list. Timing varies by dose, site, and person.

Common roles

- **Bolus (meal/correction):** rapid or short-acting
- **Basal (background):** long or ultra-long; sometimes NPH
- **Premixed:** fixed basal + bolus ratios
- **Pump / AID:** rapid-acting for basal & bolus

Safety reminders

- Never share pens or needles
- Store unopened insulin as labeled
- Track in-use pen expiry dates
- Rotate injection sites
- Clarify cloudy vs clear products before injecting

Stacking risk

Giving another correction before the previous rapid insulin has peaked can cause delayed hypoglycemia. Follow your clinician's correction spacing (often ~3–4 hours) and "insulin on board" guidance.

Educational reference only — not medical advice. Insulin regimens are individualized. Do not change doses without clinician direction.

Sick Day Rules

Illness raises stress hormones and can spike glucose — or cause lows if you can't eat. Have a plan before you need it.

Do these every sick day

1. Take basal insulin / usual meds unless told otherwise.
2. Check glucose every 2–4 hours (more if rising).
3. Check ketones if glucose ≥250 or as instructed.
4. Sip fluids often — aim for hydration.
5. Rest; ask someone to check on you.

Call your care team if

- Vomiting or diarrhea lasting >4–6 hours
- Moderate/large ketones
- Glucose stays >250–300 despite plan
- Signs of dehydration
- Fever with confusion or rapid breathing
- You can't keep fluids down
- You're unsure about medication doses

Carb & fluid ideas when appetite is low

~15G OPTIONS

- ½ cup juice
- ½ cup regular gelatin
- 6 saltines
- ½ cup applesauce
- 1 cup sports drink**

HYDRATION

- Water, broth
- Electrolyte drinks
- Ice chips / popsicles
- Sugar-free fluids if glucose high**

MEDICINE NOTES

- Know which OTC meds are OK
- Sugar-free ≠ carb-free always
- SGLT2 users: ask about ketoacidosis risk when ill
- Never stop insulin abruptly

**Choose sugared vs sugar-free fluids based on glucose and your sick-day plan.

Type 1 / insulin users

Illness can trigger DKA even if you're not eating. Continue basal insulin. Check ketones. Follow your prescribed sick-day insulin adjustments.

Emergency now

Severe abdominal pain, fruity breath, deep breathing, lethargy, or inability to keep any fluids down — seek emergency care.

Daily Foot Care Checklist

Diabetes can reduce sensation and slow healing. Small blisters or cracks can become serious infections. A two-minute daily habit protects mobility.

Every day

- [ ] Wash feet with lukewarm water & mild soap
- [ ] Dry carefully — especially between toes
- [ ] Inspect tops, soles, heels, and between toes
- [ ] Use a mirror or ask for help if needed
- [ ] Apply lotion to tops & bottoms (not between toes)
- [ ] Wear clean, dry socks (seamless if possible)
- [ ] Check shoes for pebbles, nails, rough lining
- [ ] Never walk barefoot indoors or outdoors
- [ ] Trim nails straight across; file sharp edges
- [ ] Note any redness, warmth, swelling, or drainage

Safe habits

- Test water temperature with your elbow/wrist
- Choose well-fitting shoes; break in gradually
- Alternate pairs of shoes when you can
- Keep blood flow moving — wiggle toes, ankle circles
- Don’t smoke — it worsens circulation

Avoid these

- Heating pads / hot water bottles on feet
- Medicated corn removers / razor scrapers
- Tight socks with restrictive tops
- Crossing legs for long periods
- Ignoring “minor” cuts or blisters

Call your clinician promptly for

- New ulcer, blister, or crack
- Puncture wound
- Ingrown toenail
- Red streaks or pus
- Increasing pain or swelling
- Color / temperature change
- Foot fungus that worsens
- Numbness that’s new
- Any wound not improving in 1–2 days

Annual professional check

Ask for a comprehensive foot exam at least once a year (more often with neuropathy, prior ulcers, or poor circulation). Podiatry referral can prevent serious complications.

Educational reference only — not medical advice. Seek urgent care for rapidly worsening infection, fever with a foot wound, or black/blue tissue changes.